OPERATIONAL EXCELLENCE · WHAT THE RESEARCH SAYS

Many health systems show doctors their own performance data.

In a review of many trials, clinicians who got their data followed recommended practice more often than those who did not, by about 6 percentage points on average.

But the effect varied widely, as five studies published from 2021 to 2026 show.

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THE IDEA

What is audit and feedback?

Measuring the performance of a doctor or team over a set period, then showing them the results.

A percentage point is the difference between two percentages. Going from 50% to 56% is a rise of six percentage points.

In a randomized trial, participants are split into groups by chance, and the groups' results are compared.

THE COCHRANE REVIEW

Cochrane pooled 292 randomized trials

Cochrane is an international network of researchers that gathers the studies on one question and summarizes the results.

6.2

percentage points: the average rise in recommended practice, in 177 trials comparing clinicians who got their data with those who did not.

2.7

percentage points or less: the gain in half of the results. At least a quarter showed none. This figure is calculated differently, so it cannot be compared directly with the average.

Improvement was larger among doctors who started from low performance.

ANTIBIOTIC PRESCRIBING

Clinic doctors who got their data prescribed fewer antibiotics

11%

fewer antibiotics prescribed by doctors who received their data, in 21 trials.

23%

fewer antibiotics started without need, in 16 trials.

These are primary care doctors: the clinic and health-center doctors patients usually see first. Both figures are relative reductions, measured against doctors who did not receive the data. They are not percentage points, so they cannot be compared directly with the 6.2 points on the previous slide. The researchers noted that results varied widely across trials.

MIXED RESULTS

In three other trials, not every measure improved

In Ontario, Canada, 833 doctors who examine the bowel with a camera tube were split by chance, and one group got reports on their own performance. The share of exams in which small growths (polyps) were removed rose by similar amounts in both groups. But lower performers who got a report improved more than lower performers who did not.

In seven pediatric emergency departments, leaders got a monthly report on their department. Recording of the child's vital signs, such as temperature and pulse, improved, while over the same period speed of care got worse.

In 201 bladder cancer surgery centers, recording of surgical findings improved where reports were sent, but early cancer recurrence did not differ between centers with and without reports.

WHEN WAS THE EFFECT LARGER?

The effect was larger when the data were about the doctor's own practice, not the team's

It was larger when that practice was compared with top peers or a set benchmark.

It was larger when someone from the same workplace whom the doctor knew presented the data.

It was larger when a plan with specific steps came with the report.

Most of these findings came from comparing results across trials, and the researchers said they need confirmation.

THE AUTHOR'S READING OF THE STUDIES

What can leaders do?

1

Choose an important measure where performance is still far from the goal.

2

Show each person their own numbers, have someone they know discuss them, and agree on a specific plan.

3

Watch other measures that could get worse, not only the one you target.

These are three of the six steps in the article. The author inferred them from the studies, and no study tested them together.

> **Three years from now, should performance reports in your organization be documents people forget, or a regular review of each person's numbers with a colleague they know?**

SOURCES AND LIMITS

What this evidence can and cannot tell us

In 39% of the Cochrane trials, flaws in design or conduct could make the result inaccurate; researchers call this a high risk of bias.

The review measured what health professionals did, not patient health.

None of the five study abstracts mentions a trial in an Arab country, and none tested the idea outside health care. Applying it to other sectors is the author's untested inference.

Editorial disclaimer: The views and analyses presented in this article represent the author’s professional interpretation based on the cited sources and do not necessarily represent the official positions of the organizations mentioned. For the original, complete and most current information, please consult the primary sources and official documents listed in the references.

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